



Fiona Stanley Fremantle Hospitals Group

Policy and Procedure

Older Adult Mental Health Referrals

Reference #: FSFH-MENH-PRO-0012

Scope

Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital Fremantle Hospital	Mental Health Services	Nursing, Medical, Allied Health, Administration

1. Introduction

This policy outlines the referral criteria and procedure for referring to the Fremantle Older Adult Mental Health (FOAMHS) community and inpatient Mental Health Services. All referrals are assessed for suitability and prioritised appropriately by the multidisciplinary team.

2. Terminology

BPSD	Behavioural and psychological symptoms of dementia
Transition Care Program	TCP provides short term care that seeks to optimise the functioning and independence of older people after a hospital stay. TCP is goal oriented and time limited. The aim is to provide older people with an option to improve their independence (restorative pathway) or to provide support to secure permanent aged residential care (residential pathway).

3. Policy

Any emergency/urgent referrals requiring a response time less than 72 hours must be directed to Fremantle Hospital Assessment and Treatment Team (ATT) or the nearest Emergency Department (ED).

3.1. Inclusion criteria

Patients/consumers referred to FOAMHS must be:

- aged ≥ 65 years (55 years for Aboriginal persons who self-identify as older adult)

- reside in the FSFHG mental health catchment area and are experiencing:
 - Late onset mental illness (or persons not active with adult Mental Health Services for 2 years or more)
 - Dementia diagnosis with behaviour disturbance (can include persons younger than 65 when these symptoms present)
 - Symptoms of mental illness and whose needs are best met in older adult care environment.
- In the event an adult consumer is admitted to the OAMH Inpatient ward for environmental reasons/access to appropriate equipment, they will be discharged to the care of the originating adult team. Refer to the Older Adult Graduate Referral Admission Process Guideline.
- Requires a completed organic screen (within the previous three months) sent with referral or within 2 weeks of referral)
- The organic screen will include:

MSU	FBC	LFT
TFT	U & Es	Random BSL
Calcium	B12	Folate
Drug levels (lithium/Sodium Valproate)		
Neuroimaging reports (if avail)		

3.3. Exclusion Criteria

Where other specialist services are identified as more appropriate. These conditions include:

- Intellectual disability
- Acute physical illness/ delirium
- Acquired brain injury
- Neuro-cognitive deficits in people suffering from long term/ major psychiatric illnesses.
- Younger adults who have been seen by ACAT and are residing in Residential Aged care Facilities.

Also excluded are referrals for:

- forensic assessments
- expert opinions to courts
- managing primary issues of:
 - accommodation
 - hoarding

- self-neglect in the absence of formally diagnosed acute major mental health issues
- nursing home placement
- alcohol and drug use.

4. Procedure

Clinical governance of a patient's treatment remains with the referring team until the referral is accepted and a formal agreement is reached that patient care will be transferred to FOAMHS

4.1. Community referrals

Referrers will:

- General Practitioners / external services will refer using the:
 - [Mental Health Service – Adult and Older Adult GP Referral Form](#)
- Internal referrers will complete eReferral and wherever possible include a documented:
 - Mental Health Assessment Form
 - Risk Assessment and Management Plan
 - Functional Assessment (Older Adults)

FOAMHS will:

- Triage referrals based on referral contents, existing notes in the DMR and PSOLIS, contact with the referrer and the patient and their family where appropriate.
- Present triaged referrals to the FOAMHS daily intake meeting (held Monday - Friday at 9.00am) by the Triage officer for background and context
- Make an MDT decision to accept or decline referral
- Inform the referrer of the meeting outcome. If the referral is declined, alternative destinations may be suggested
- If it is determined that an inpatient admission is clinically indicated for a patient refer to Waitlisting for Older Adult Mental Health Admission procedure.

4.2. Transfer of care

Transfer of care from an external outpatient Older Adult Mental Health Service requires:

- 2 week follow up and joint review / handover (Care Coordinator – Care Coordinator from ward /community where possible) with FOAMHS prior to transfer of care when community follow up is required.

- a Mental Health Care Transfer Summary, Risk Assessment and Management Plan and updated Treatment Support Discharge Plan will be provided to the receiving team, documenting details of required medications and ongoing care plan.
- if readmission to the inpatient setting is required, this will be directed to the original catchment inpatient team within the agreed 3-month readmission period, or for high dependency unit patients within a 12-month readmission period.

4.3. Mental Health Transition Care Program (TCP) Bullcreek.

- The community team initiating the referral will continue to provide clinical follow-up care as required for the duration of the patient's TCP stay.

4.4. Graduate referrals

- Graduate referrals will be considered and processed in line with the Older Adult Graduate Referral Admission Process.

5. Compliance/performance monitoring

- FOAMHS Team Leader will monitor compliance with referral criteria
- Any referrals which do not meet the referral criteria will be received not admitted (RNA) and the outcome communicated to the referrer.

6. Related policy documents

WA Health

- [WA Transition Care Program Operational Guidelines 2022](#)

SMHS:

- [Care coordination in mental health](#) (SMHS :13)

FSFHG:

- Clinical Documentation (FSFH-HW-POL-0039)
- Older adult graduate referral admission process (FSFH-MENH-GUI-0005)
- Urgent Mental Health Assessment for persons 65 years and over (FSFH-MENH-GUI-0007)
- Waitlisting for Older Adult Mental Health Admission (FSFH-MENH-PRO-0010)
- Clinical risk assessment and management (FSFH-HW-POL-0017)

7. Related standards

NSQHS Standards:

- Clinical Governance
- Communicating for Safety

Chief Psychiatrist Standards for Clinical Care

- Transfer of Care

8. Authorisation

EXECUTIVE SPONSOR: Service Director – Service 5					
Version	Date Issued	Compiled/Revised By	Committee/Consumer Group Consulted	Endorsed By	Revision due
1	12/2020	Program Manager, Older Adult	MH SQR Committee	MH SQR Committee (Single Speciality Process)	12/2023
2	09/2023	Team Leader, Older Adult		MH SQR Committee (Single Speciality Process)	10/2027